RECURRENT
TEMPLATE FOR APPEAL
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[Date]

ATTN: [Dept Name]
[Name of Insurance Company]
FAX #: [fax #]

RE: [L1/L2/reconsideration/claim appeal/appeal type] of a denied Prior Authorization/Pre-Determination request for PSMA-Targeted PET Imaging with PYLARIFY® (piflufolastat F 18) Injection

ICD10 Diagnosis Code(s):
CPT / HCPCS Code(s):

Patient Name:
Subscriber ID #:
Group #:
Date of Birth (DOB):
[reference #/case # from denied request]
Date of service: (if a claim appeal)
Billed amount: (if a claim appeal)
Claim #: (if a claim appeal)

Dear Medical Reviewer:

I am writing to request a [L1/L2/reconsideration/appeal type] of a denied prior authorization/pre-determination for Prostate-Specific Membrane Antigen (PSMA) Targeted PET imaging with PYLARIFY® (piflufolastat F 18) injection for my patient [patient name] who has suspected recurrent prostate cancer based on an elevated serum PSA level.

Per denial letter dated (date), (payer name) denied the request stating, *"list the denial reason given here"*. We firmly disagree with this decision for the following reasons:

- **The NCCN recognizes the role of PSMA tracers as a front-line imaging tool for patients with biochemical recurrence of prostate cancer.**
- **PYLARIFY® is an FDA approved tracer for imaging prostate cancer**
- **The pivotal CONDOR study reported that PYLARIFY achieved the primary endpoint of correct localization rate: 85% - 87% across all 3 readers**
- **The pivotal CONDOR study reported that in 64% of patients, PYLARIFY imaging resulted in changes in intended patient management.**
- **The pivotal OSPREY study reported high levels of sensitivity and positive predictive value compared to histopathology in patients with advanced disease.**
- **Reference any other reason why you disagree with denial**

(Please include patient details that support appeal in the paragraph below (e.g. date of prostate cancer diagnosis, prior treatment history, PSA response to prior treatment, current PSA) and then complete this section with the provided

language)

Patient has been diagnosed with prostate cancer on (date).

- Prior treatments included (i.e., prostatectomy, radiation, ADT, etc) on date(s)
- PSA response (nadir) to prior treatment included…
- He now has suspected recurrence of prostate cancer with an elevated PSA of x…

(Refer back to payer policies for specific list of medical criteria to customize above)

Based on this assessment and current clinical guidelines, this patient has evidence of recurrent prostate cancer.

PSMA targeted PET imaging will assist in determining the location and the extent of his recurrent disease. The purpose of using PSMA-targeted PET imaging with piflufolastat F 18 injection is to detect and locate metastatic spread of cancer as PYLARIFY binds to the prostate specific membrane antigen protein found on the surface of over 90% of prostate cancer cells. Based on the results of PSMA-targeted PET imaging with PYLARIFY, we believe we will be able to recommend a more personalized and appropriate management plan for (name of patient).

Per review of your medical policy (enter #/title), (enter insurance co/RBM name) provides positive coverage for other radiopharmaceuticals, and we feel the same consideration should be given for PYLARIFY radiopharmaceutical agent.

The NCCN recognizes the role of PSMA tracers as a front-line imaging tool for patients with biochemical recurrence of prostate cancer.

NCCN Clinical Practice Guidelines in Oncology – Prostate Cancer Version 4.2024 – May 17, 2024 indicates:

- F-18 piflufolastat (PYLARIFY®) can be considered as an alternative to standard imaging of bone and soft tissue for initial staging, the detection of biochemically recurrent disease, and as a workup for progression.
- Because of the increased sensitivity and specificity of PSMA-PET tracers for detecting micrometastatic disease compared to conventional imaging (eg, CT, bone scan) at both initial staging and biochemical recurrence (BCR), the Panel does not feel that conventional imaging is a necessary prerequisite to PSMA-PET and that PSMA-PET/CT or PSMA-PET MRI can serve as an equally effective, if not more effective front line imaging tool for these patients[1]

PYLARIFY® is an FDA approved tracer for imaging prostate cancer.

Approved by the FDA as of May 26, 2021, the labeled indication is as follows:

> Piflufolastat F 18 injection is a radioactive diagnostic agent indicated for positron emission tomography (PET) of prostate-specific membrane antigen (PSMA) positive lesions in men with prostate cancer:
>
> - with suspected metastasis who are candidates for initial definitive therapy.
> - with suspected recurrence based on elevated serum prostate-specific antigen (PSA) level.

The pivotal CONDOR study reported that in 64% of patients, PYLARIFY imaging resulted in changes in intended patient management.

[1] NCCN Clinical Practice Guidelines in Oncology – Prostate Cancer Version 4.2024 – May 17, 2024. Available at: https://www.nccn.org/login?ReturnURL=https://www.nccn.org/professionals/physician_gls/pdf/prostate.pdf

The CONDOR clinical study[2] was a Phase 3 prospective multicenter study of PYLARIFY (piflufolastat F 18) injection in 208 men with biochemically recurrent prostate cancer and uninformative standard imaging. The median baseline PSA was 0.8 ng/mL (range: 0.2-98.4 ng/mL), all patients underwent a PSMA-targeted PET imaging scan with PYLARIFY (piflufolastat F 18) injection. The disease detection rate range was 59% to 66% (≥ 1 lesion detected per patient by piflufolastat F 18 PET/CT by 3 central readers). The correct disease localization rate (CLR) was 84.8%-87.0% (lower bound of 95% CI: 77.8%-80.4%).

In the CONDOR study, although the impact on patient management was not prospectively evaluated, 64% of subjects had a change in intended management of the disease, with 79% of the changes coming after a positive PSMA-targeted PET imaging with piflufolastat F 18 injection scan while 21% of the changes followed a negative piflufolastat F 18 injection scan highlighting the utility and actionability of the scan on treatment recommendations.[3]

The most frequent changes to treatment management plans after PYLARIFY-PET/CT imaging included salvage local therapy that was either supplemented or replaced by systemic therapy (n = 58; 28.3%), and systemic therapy to salvage local therapy (n = 43; 21.0%).[4]

The suboptimal diagnostic performance of conventional imaging in patients with prostate cancer often leads to ineffective under-treatment or unnecessary over-treatment. The selection of appropriate therapies is contingent upon the accurate assessment of the patient's "true" burden of disease.

It is estimated that 30-40% of patients will fail initial therapy and require additional treatment with many of these patients harboring occult metastatic disease[5,6]. PSMA-targeted PET imaging improves disease assessment, which reduces of the risk of undertreatment (leading to disease spread) and overtreatment (leading to unnecessary toxicity or surgery).

Given PSMA-targeted PET imaging with piflufolastat F 18 injection's demonstrated ability to accurately detect prostate cancer lesions, both at initial diagnosis and at recurrence of the disease after treatment failure, utilization of piflufolastat F 18 has been shown to enable and improve disease assessment for patients for patients across the spectrum of prostate cancer.

The pivotal OSPREY study reported high levels of sensitivity and positive predictive value compared to histopathology in patients with advanced disease.

In OSPREY cohort B, sensitivity and PPV of ^{18}F-DCFPyL-PET/CT for detecting sites of prostate cancer metastasis or locoregional recurrence were evaluated. (93 evaluable patients, median prostate specific antigen 11.3 ng/ml), median sensitivity and positive predictive value for extraprostatic lesions were 95.8% (87.8%e99.0%) and 81.9% (73.7%e90.2%), respectively.[7]

For the reasons stated above, I strongly believe that PSMA-targeted PET imaging with piflufolastat F 18 injection is appropriate and required for [patient name] and should be a covered benefit.

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[2] Morris MJ, Rowe SP, Gorin MA, Saperstein L, Pouliot F, Josephson D, Wong JYC, Pantel AR, Cho SY, Gage KL, Piert M, Iagaru A, Pollard JH, Wong V, Jensen J, Lin T, Stambler N, Carroll PR, Siegel BA; CONDOR Study Group. Diagnostic Performance of ^{18}F-DCFPyL-PET/CT in Men with Biochemically Recurrent Prostate Cancer: Results from the CONDOR Phase III, Multicenter Study. Clin Cancer Res. 2021 Jul 1;27(13):3674-3682. doi: 10.1158/1078-0432.CCR-20-4573. Epub 2021 Feb 23. PMID: 33622706.

[3] Morris, et al.

[4] Morris, et al.

[5] Medical News Today. How nerves fuel prostate cancer growth. Available at: https://www.medicalnewstoday.com/articles/319789

[6] Paller CJ et al. *Hematol Oncol Clin North Am.* 2013;27(6):doi:10.1016/j.hoc.2013.08.005

[7] Pienta KJ, Gorin MA, Rowe SP, Carroll PR, Pouliot F, Probst S, Saperstein L, Preston MA, Alva AS, Patnaik A, Durack JC, Stambler N, Lin T, Jensen J, Wong V, Siegel BA, Morris MJ. A Phase 2/3 Prospective Multicenter Study of the Diagnostic Accuracy of Prostate Specific Membrane Antigen PET/CT with ^{18}F-DCFPyL in Prostate Cancer Patients (OSPREY). J Urol. 2021 Jul;206(1):52-61. doi: 10.1097/JU.0000000000001698. Epub 2021 Feb 26. PMID: 33634707.

Thank you for your time and consideration. If you have any questions and/or concerns, regarding the procedure or this patient's medical condition, please do not hesitate to contact me at the number listed below.

We look forward to [payer name]'s timely, positive response.

Sincerely,

[MD Name]
[Practice Group Name]
[Street Address]
[City/Town, State Zip]
Phone #:
Fax #:
TIN:
NPI #:

Imaging/Hospital Facility info:
[Facility Name]
[Street Address]
[City/Town, State Zip]
Phone #:
Fax #:
TIN:
NPI #:

Enclosures

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